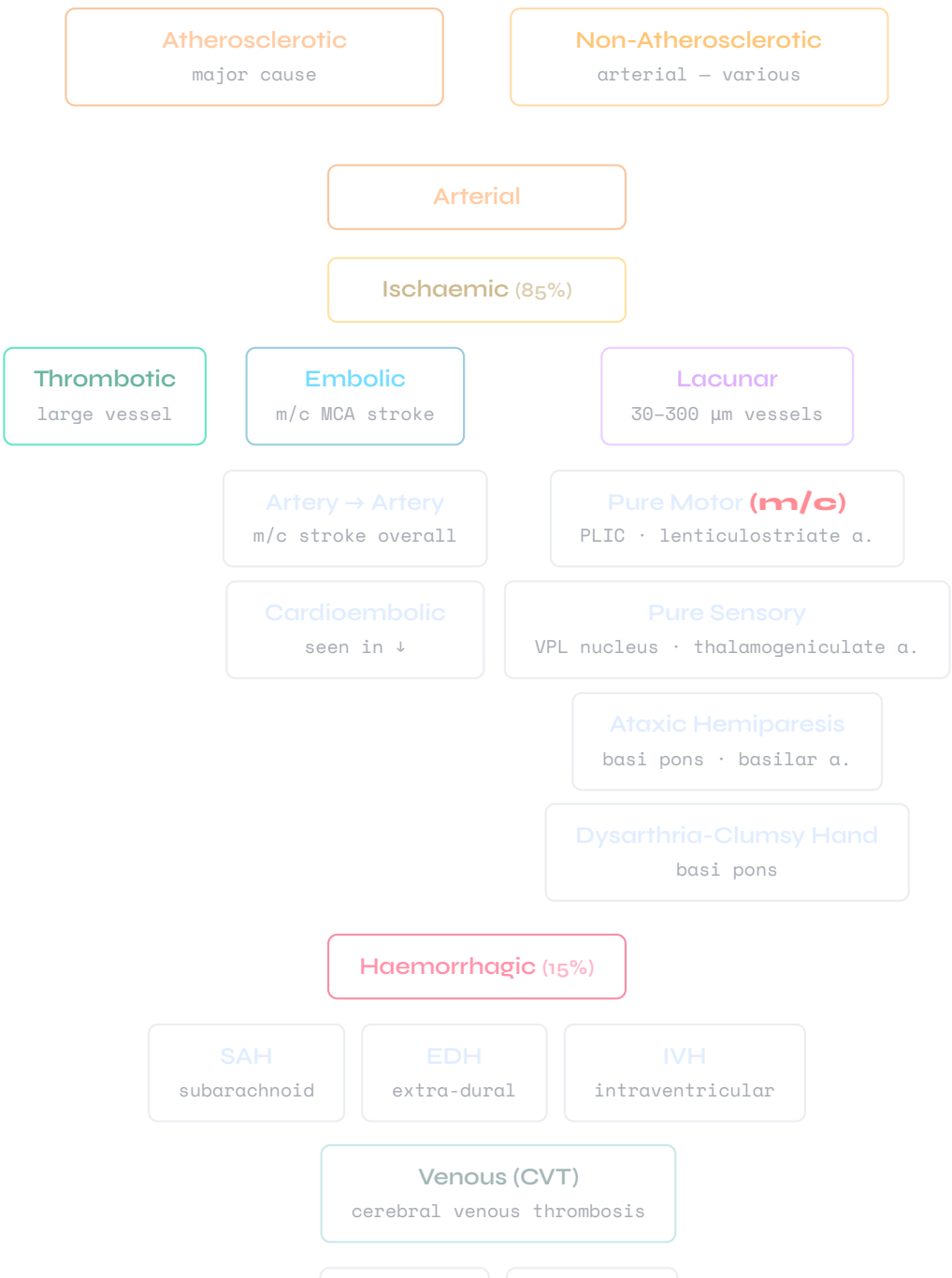


Middle Cerebral Artery Stroke

CONCEPT MAP V2 – MARROW-ALIGNED · ETIOLOGY & TYPES

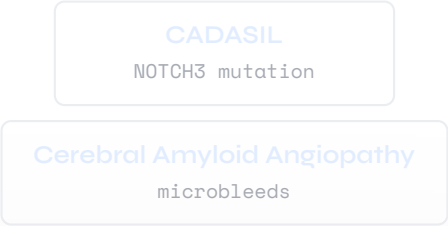
Core Arterial Venous Ischemic Hemorrhagic Lacunar MCA Types

ETIOLOGY – BROAD CLASSIFICATION



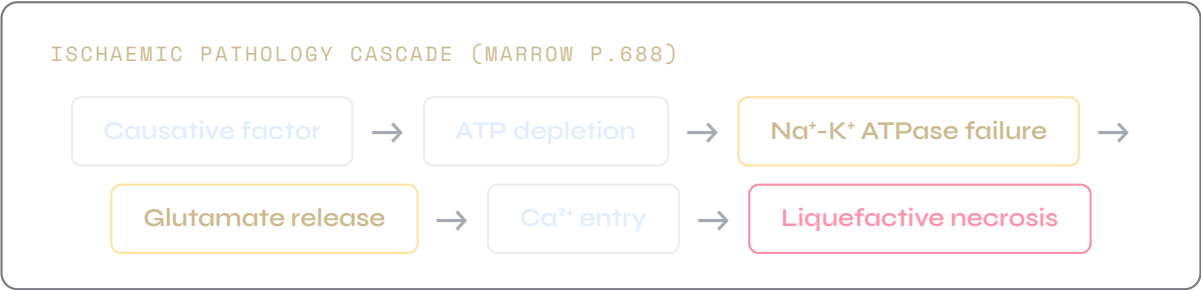


WHITE MATTER CONDITIONS



MCA STROKE
~70% of all strokes · lateral cortex + BG + IC

MCA STROKE TYPES BY TERRITORY



THROMBOTIC VS EMBOLIC STROKE (MARROW COMPARISON)

FEATURE	THROMBOTIC	EMBOLIC
Onset	Stroke in evolution (progressive)	Hyperacute (maximal at onset)

Features	↑ Headache	LOC / seizures
Weakness	↑ over 48–72 hrs	Full at onset
Clot type	Fibrin clot (white)	Red clot
Transformation	–	Hemorrhagic transformation
Cortical signs	Variable	Present (+)
Recovery	Gradual	Rapid (partial) possible

CLINICAL FEATURES BY HEMISPHERE

DOMINANT (L) MCA

Aphasia
(Broca/Wernicke/Global)

Right hemiplegia

Right hemisensory loss

Right homonymous
hemianopia

Alexia / Agraphia

Face + Arm > Leg

NON-DOMINANT (R) MCA

Left hemispatial
neglect

Anosognosia

Constructional apraxia

Aprosodia

Left homonymous
hemianopia

Left hemiplegia

GAZE DEVIATION

Eyes toward lesion
(cortical)

"Looking at the stroke"

Frontal eye field
damage

Pontine: eyes away from
lesion

EARLY CT SIGNS (MCA)

Hyperdense MCA sign
(≥6h) – earliest

MCA dot sign

Insular ribbon sign (6–
48h)

Obscuration of
lentiform nucleus (8–
24h)

ACUTE MANAGEMENT



hover any node for detail · Marrow 8.0 Medicine pp.687-690 · MCA Stroke Concept Map v2